

Duspatalin[®] 135 mg coated tablets

135 mg mebeverine hydrochloride



Read all of this leaflet carefully before you start taking this medicine.

Keep this leaflet. You may need to read it again. If you have questions not answered by this pamphlet, please ask your doctor or pharmacist. This medicine has been prescribed to you personally and you should not pass it on to others. It may harm them, even if their symptoms are the same as yours.

Duspatalin 135 mg coated tablets are round, white, sugar-coated tablets for oral administration (to be taken by mouth), containing 135 mg mebeverine hydrochloride per tablet.

Excipients (non-medicinal ingredients):

Core: lactose monohydrate, starch (potato), povidone, talc, magnesium stearate

Coating: talc, sucrose, gelatine, acacia, carnauba wax

Indications

Symptomatic treatment of abdominal pain and cramps, bowel disturbances and intestinal discomfort related to irritable bowel syndrome.

Treatment of gastro-intestinal spasm secondary to organic disease.

Dosage and administration

Adults

Take one tablet three times daily, approximately 20 minutes before a meal.

Swallow the tablets with at least 100ml water, do not chew.

Always take Duspatalin exactly as your doctor has prescribed. If you have any questions, check with your doctor or pharmacist.

If you forget to take your tablet (s), do not take a double dose to compensate for it. If you require further information, please ask your doctor or pharmacist for advice.

Paediatric population

Duspatalin 135mg tablets are not recommended for use in children and adolescents below 18, due to insufficient data on safety and efficacy.

Contraindications

Hypersensitivity to the active substance or to any of the excipients listed above.

Warnings and special precautions for use

Since Duspatalin coated tablets contain lactose, patients with rare hereditary problems of galactose intolerance, the Lapp lactase deficiency or glucose-galactose malabsorption should not take this medicine.

Since Duspatalin coated tablets contain sucrose, patients with rare hereditary problems of fructose intolerance, glucose-galactose malabsorption or sucrase-isomaltase insufficiency should not take this

medicine.

Interactions with other medications

No interaction studies have been performed with the exception of alcohol. In vitro and in vivo-studies in animals have demonstrated the absence of any interaction between Duspatalin and ethanol.

Fertility, pregnancy and lactation

Pregnancy

There are no or limited amount of data from the use of Duspatalin in pregnant women. Animal studies are insufficient with respect to reproductive toxicity (see Preclinical Safety Data). Duspatalin is not recommended during pregnancy.

Lactation

It is unknown whether mebeverine or its metabolites are excreted in human milk. The excretion of mebeverine in milk has not been studied in animals. Duspatalin should not be used during breast-feeding.

Fertility

There are no clinical data regarding impact on male or female fertility; however, available animal studies do not indicate harmful effects of trademark (see Preclinical Safety Data).

Effects on ability to drive and use machines

No studies on the effects on the ability to drive and use machines have been performed. The pharmacodynamic and pharmacokinetic profile as well as post-marketing experience do not indicate any harmful effect of mebeverine on the ability to drive or to use machines

Undesirable effects

The following adverse events have been reported spontaneously during post-marketing use. A precise frequency cannot be estimated from available data.

Allergic reactions mainly but not exclusively limited to the skin have been observed.

Skin and subcutaneous tissue disorders:

Urticaria, angioedema, face edema, exanthema

Immune system disorders:

Hypersensitivity (anaphylactic reactions)

Reporting of suspected adverse reactions:

Reporting suspected adverse reactions after authorisation of the medicinal product is important. It allows continued monitoring of the benefit/risk balance of the medicinal product. Healthcare professionals are asked to report any suspected adverse reactions via the local authorities.

Overdose

Symptoms

Theoretically, CNS excitability may occur in cases of overdose. In cases where mebeverine was taken in overdose, symptoms were either absent or mild and usually rapidly reversible. Observed symptoms of overdose were of neurological and cardiovascular nature.

Treatment

No specific antidote is known and symptomatic treatment is recommended. Gastric lavage should only be considered in case of multiple intoxication discovered within about one hour. Absorption reducing measures are not necessary.

Pharmacodynamics

Pharmacotherapeutic group: Synthetic anticholinergics, esters with tertiary amino group.

ATC-Code: A03AA04

Mechanism of action and pharmacodynamic effects

Mebeverine is a musculotropic antispasmodic with a direct effect on the smooth muscle of the gastrointestinal tract, without affecting normal gut motility.

The exact mechanism of action is not known, but multiple mechanisms, such as a decrease in ion channel permeabilities, blockade of noradrenaline reuptake, a local anesthetic effect, changes in water absorption might contribute to the local effect of mebeverine on the gastrointestinal tract. Via these mechanisms mebeverine has antispasmodic effects leading to normalization of gut motility without exerting a permanent relaxation of smooth muscle cells in the gastrointestinal tract (so called hypotonia). Systemic side-effects as seen with typical anti-cholinergics are absent.

Clinical efficacy and safety

The clinical efficacy and safety of different formulations of mebeverine was evaluated in more than 1500 patients. Considerable improvements in the predominant symptomatology of irritable bowel syndrome (e.g. abdominal pain, stool characteristics) were generally observed in reference or baseline-controlled clinical studies.

Mebeverine were generally safe and well tolerated in the recommended dose regimen.

Paediatric population

Clinical trials with the tablet or capsule formulations have been performed in adults only.

The dosing schedule for the tablet formulation was calculated based on the consistent safety and favourable tolerability of mebeverine.

Pharmacokinetics

Absorption:

Mebeverine is rapidly and completely absorbed after oral administration of tablets.

Distribution:

No significant accumulation occurs after multiple doses.

Biotransformation:

Mebeverine hydrochloride is mainly metabolized by esterases, which split the ester bonds into veratric acid and mebeverine alcohol.

The main metabolite in plasma is DMAC (demethylated carboxylic acid).

The steady state elimination half-life of DMAC is 2.45 h. During multiple dosing C_{max} of DMAC for the coated tablets with 135 mg is 1670 ng/ml and t_{max} is 1 h.

Elimination:

Mebeverine is not excreted as such, but metabolized completely; the metabolites are excreted nearly completely.

Veratric acid is excreted into the urine, mebeverine alcohol is also excreted into the urine, partly as the corresponding carboxylic acid (MAC) and partly as the demethylated carboxylic acid (DMAC).

Paediatric population

No pharmacokinetic studies have been conducted in children with any formulation of mebeverine.

Preclinical Safety Data

Effects in repeat-dose studies after oral and parenteral doses were indicative of central nervous involvement excitation with behavioral, mainly tremor and convulsions. In the dog, the most sensitive species, these effects were seen at oral doses equivalent to 3 times the maximum recommended clinical dose of 400mg/day based on body surface area (mg/m²) comparisons.

The reproductive toxicity of mebeverine was not sufficiently investigated in animal studies. There was no indication of teratogenic potential in rats and rabbits. However, embryotoxic effects (reduction in litter size, increased incidence of resorption) were noticed in rats at doses equivalent to twice the maximum daily clinical dose. This effect was not observed in rabbits.

No effects on male or female fertility were noted in rats at doses equivalent to the maximum clinical dose.

In conventional in vitro and in vivo genotoxicity tests mebeverine was devoid of genotoxic effects. No carcinogenicity studies have been performed.

Incompatibilities

Not applicable.

Shelf life and storage conditions

This product can be stored for up to 5 years.

Do not use the medicine after the expiry date stated on the carton.

Do not store above 30°C.

Store in the original package.

Keep this medicine out of the reach and sight of children.

Pack sizes

Duspatalin 135 mg coated tablets are supplied in packages containing 9, 10, 20, 30, 50, 60, 90, 100, 250, 500, or 1000 tablets per pack (not all pack sizes may be marketed).

The press-through blister strips are made of PVC and aluminum lidding foil.

Further information

Any unused product or waste material should be disposed of in accordance with local requirements.

The information in this leaflet is limited. For further information, please contact your doctor or pharmacist.

Date of information

Feb 2025

Manufactured by

Mylan Laboratories SAS
Route de Belleville
Lieu-dit Maillard
01400 Châtillon-sur-Chalaronne
France

SOLID 1000308976 v5.0